

整合身體與認知活動水平的綜合評估工具

後設資料

想法編號: seed-theory-1=>1=>3=>1

新穎度: 46.8%

最大相似度: 53.2% (近似於: “Middle-aged Females’Self-evaluation of Their Daily Activity Levels”)

群集: 7

競賽得分: 4 勝

方法論契合度: 2/5

問題陳述

隨著全球老年人口的增加，身體活動不足和認知功能下降已成為主要的公共健康問題。現有的研究主要集中於身體活動或認知能力的單獨評估，缺乏對兩者整合影響的系統性評估。這一問題的重要性在於，身體與認知活動之間的互動不僅影響老年人的生活質量，還可能影響其健康狀況和疾病風險。因此，開發一種綜合評估工具，以便全面監控身體和認知活動的水平，對個性化的干預策略至關重要，能夠幫助臨床護理、公共健康政策制定以及改善老年人的整體健康狀態。

現有方法

目前有幾種量表和方法可以測量身體活動和認知功能，例如國際身體活動量表（IPAQ）及各種認知評估量表。然而，這些方法多數是分開進行的，無法同時評估身體活動與認知任務的相互影響。此外，許多現有工具的信效度在特定人群，尤其是非西方文化背景的老年人中尚未得到充分驗證。這使得這些工具在實際臨床應用中，尤其是在台灣，面臨著有效性和準確性的挑戰。

研究動機

現有的方法大多無法充分捕捉老年人日常生活中身體與認知活動的動態交互，這使得由於缺乏綜合性評估工具而導致的個性化干預方案在設計與實施上存在困難。此外，許多量表未考慮到文化特性與語言差異，這限制了其在台灣地區的應用。因此，提出一個創新的整合評估工具，不僅能設計出能同時測量身體活動與認知功能的指標，還能在這一基礎上分析兩者之間的相互影響，具有巨大的潛在價值。

提出方法

本研究提議開發一個名為「整合型身體與認知活動評估工具」的測量系統。該工具旨在同時評估老年人的身體活動頻率、強度及其在進行認知任務時的表現。我們的工具將包含以下幾個關鍵步驟：首先，進行文獻綜述，篩選並整合現有的測量指標，然後通過專家訪談和焦點小組討論進一步確定工具的內容和結構，確保其文化適應性。接下來，設計一系列的身體活動與認知任務，包括平衡、記憶、注意力等多重任務，並利用可穿戴設備捕捉身體活動數據。這些數據將與參與者的認知任務表現相結合，分析身體與認知活動的互動效果。我們會利用多元回歸分析和結構方程模型，來揭示身體與認知活動對老年人健康影響的潛在機制。

實驗計畫

實驗計畫將包括以下步驟：第一步，招募 200 名 65 歲以上的社區老年人，根據台灣的人口數據進行分層抽樣。第二步，使用焦點小組和專家評估方法，確定最終的評估工具內容。第三步，所有參與者將接受為期 6 個月的身体與認知活動訓練，並每月進行評估，使用可穿戴設備（如加速度計）獲取實際的身體活動數據，並設計認知任務的測試。第四步，將使用 SPSS 和 AMOS 軟件進行數據分析，計算每個

變數的信效度和結構方程模型的適合度，最終的評估指標包括身體與認知活動的相關性及其對健康結果的影響。成功的結果將包括身體活動與認知任務表現的顯著正相關，以及總體健康狀況的改善證據。

可信新穎度 — 最接近的真實論文

- Middle-aged Females' Self-evaluation of Their Daily Activity Levels (53.2% · airtiti.AL, 2015) — 區隔明確: 該研究主要集中於中高年女性的日常生活活動量自我評估，並未探討身體活動與認知功能的整合評估，因此缺乏對兩者互動影響的系統性分析。
- Comparison of Nutritional Habits, Physical Activity Levels and Quality of Life among Normal Cognition Elderly Individuals Living in Nursing Homes or at Their Residence (52.5% · airtiti.AL, 2019) — 區隔明確: 這篇文章比較了居住在護理院和家庭的老年人的營養習慣、身體活動水平和生活質量，但並未同時評估身體活動和認知功能的互動影響，因此缺乏對兩者整合的評估工具的開發。
- 老年人身體活動量表中文版之信度與效度 (52.2% · airtiti.AL, 2013) — 區隔明確: 該研究專注於老年人身體活動量表的信度與效度檢驗，並未涉及認知功能的評估或兩者之間的互動，因此缺乏對身體與認知活動整合影響的分析。

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61. Physical activity and public health: A recommendation from the CDC and ACSM
62. Cognitive laboratory approach to designing questionnaires for surveys of the elderly

社區基礎生態干預對老年人自我照顧和身體活動的推廣效果研究

後設資料

想法編號: seed-theory-1=>1=>3=>2

新穎度: 43.7%

最大相似度: 56.3% (近似於: “Comparison of Nutritional Habits, Physical Activity Levels and Quality of Life among Normal Cognition Elderly Individuals Living in Nursing Homes or at Their Residence”)

群集: 5

競賽得分: 3 勝

方法論契合度: 2/5

問題陳述

隨著全球老年人口的增加，老年人面臨的健康挑戰也日益增加，包括慢性疾病管理和生活質量的提升。然而，許多老年人缺乏足夠的身體活動和自我照顧的能力，這對他們的健康和福祉構成了威脅。現有健康促進策略多數集中在個體行為的改變，卻忽視了社區環境和社會支持系統對老年人健康行為的影響。因此，探討社區基礎的生態干預是否能有效促進老年人自我照顧和身體活動，進而提升其生活質量和慢性病的管理至關重要。

現有方法

目前的研究方法多數集中於個體層面的行為干預，例如使用健康教育推廣和運動指導。然而，這些方法往往忽略了社區環境的影響，導致干預效果有限。儘管一些研究已開始探索社區支持的角色，但缺乏系統性的方法來整合社區資源及變量，以促進老年人的健康行為。此外，許多現有的方法在評估和適用於老年人時缺乏充分的信效度驗證，因此還需要更全面的研究來填補這一空白。

研究動機

現有方法主要以個體為中心，常常忽略了環境和社會支持的重要性。這使得干預效果在實際應用中受到限制，而老年人群體在社區中的生活方式及健康行為往往受到周遭環境的深刻影響。因此，本研究提出的生態干預方法將系統地整合社區資源，包括社會支持和環境設計，旨在創造一個良好的健康促進氛圍。這樣的干預方式不僅能提升老年人的身體活動水平，還能增強社區的凝聚力，促進老年人之間的互動，從而更全面地改善其健康狀況。

提出方法

本研究將採用社區基礎的生態干預方法，通過整合社區資源，促進老年人自我照顧及身體活動。具體方法包括：首先，組建由社區成員、醫療專業人員以及志願者組成的干預小組，進行定期會議以確認當地老年人的需求。其次，設計多樣化的活動計劃，如健康講座、體驗式運動課程、社交活動等，並鼓勵老年人參加，以提升他們的參與度和動機。此外，該計劃將設立社區支持中心，提供資源及輔導，幫助老年人克服參加活動的障礙。研究將隨機選取若干社區，進行基線及干預期間的健康行為和生活質量測量，並透過質量訪談評估參與者的經驗和反饋。整體而言，這一方法將針對社區多重層面進行介入，從而更有效地促進老年人的健康行為及生活質量。

實驗計畫

本實驗計畫分為四個主要步驟：首先，選擇台灣各地的老年人社區進行隨機抽樣，收集基線數據，包括健康狀況、身體活動量 and 自我照顧能力等。其次，組織社區干預小組，進行為期六個月的生態干預措施，包括定期的健康講座和體育活動，每月進行一次社區聚會，並設置支持系統以提供持續的激勵。第三，使用國際身體活動量表台灣中文版來評估參與者在干預前後的身體活動情況及自我照顧能力。最後，通過統計分析比較干預前後的數據，並評估社區干預對老年人生活質量的影響。成功的指標將包括身體活動量的提升、自我照顧能力的增強，以及生活質量的改善。

可信新穎度 — 最接近的真實論文

- Comparison of Nutritional Habits, Physical Activity Levels and Quality of Life among Normal Cognition Elderly Individuals Living in Nursing Homes or at Their Residence (56.3% · airtiti.AL, 2019) — 區隔明確: 本研究專注於老年人在護理機構與居住環境中的營養習慣、身體活動水平及生活質量的比較，未涉及社區基礎的生態干預方法或社會支持系統的影響，而本提案則強調社區干預對老年人自我照顧及身體活動的促進，並整合社區資源以提升生活質量。
- Management of falls in community-dwelling older adults: clinical guidance statement from the Academy of Geriatric Physical Therapy of the American Physical Therapy Association. (52.3% · airtiti.PubMed, 2015) — 區隔明確: 該研究專注於老年人跌倒的管理，提供臨床指導，並未探討社區基礎的生態干預如何影響老年人的自我照顧和身體活動，而本提案則針對社區環境的介入進行研究，並強調社會支持系統的作用。
- Self-affirmation enhances the intention to improve physical inactivity through health risk messages (51.1% · airtiti.AL, 2021) — 區隔明確: 這項研究探討自我肯定如何影響人們改善健康行為的意圖，並未涉及社區基礎的生態干預或對老年人身體活動的具體促進措施，而本提案則專注於社區資源的整合及其對老年人健康行為的影響。

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使用 IPAQ 評估社區層面身體活動政策的有效性

後設資料

想法編號: seed-domain-1=>3=>2=>3

新穎度: 46.6%

最大相似度: 53.4% (近似於: “Development and Validation of the Papua New Guinea Physical Activity Questionnaire”)

群集: 8

競賽得分: 3 勝

方法論契合度: 4/5

問題陳述

身體活動不足是導致全球健康問題的重要因素，台灣在這方面的狀況日益嚴重。雖然出台了許多旨在促進身體活動的政策，但對這些政策在社區層面的實施效果卻缺乏系統性的評估。現有的測量工具往往無法全面反映社區居民的身體活動情況，以及這些活動與健康政策的實際對應。因此，本研究將利用國際身體活動量表台灣版本（IPAQ-Taiwan）作為主要工具，對社區層面的身體活動政策進行深入的評估，確定其有效性並提出改進建議。這一研究具有重大意義，因為它不僅能夠為當前的健康政策提供實證支持，還能促進公共健康的未來發展。

現有方法

目前許多研究使用了各種量表來評估身體活動水平，如國際身體活動量表（IPAQ）和其他自評問卷。然而，這些方法有其局限性，主要是缺乏針對台灣社區的地方性調整與驗證，且往往忽視社區文化及環境因素對身體活動的影響。此外，傳統的評估方式多數為一次性調查，無法長期追蹤政策的影響，這使得研究結果難以反映社區的實際狀況和需求。

研究動機

現有的身體活動評估方法在許多情況下無法充分捕捉到社區居民的活動模式及其對健康政策反應的多樣性。特別是在台灣，社會文化的獨特性及居住環境的差異使得通用的測量工具往往無法完全適用。因此，我們的研究計劃通過 IPAQ-Taiwan 版本的本土化，結合社區深入訪談和焦點小組討論，來獲取更具社會文化相關性的數據。我們相信這種方法不僅能準確反映社區居民的身體活動情況，還能揭示出他們對健康政策的理解與接受程度，從而更有效地指導未來的政策制定。

提出方法

本研究的主要方法是採用 IPAQ-Taiwan 版本進行社區層面的身體活動評估。我們將進行以下幾個步驟：首先，與當地公共衛生機構和社區組織合作，選擇幾個不同的社區作為研究對象，涵蓋城市和鄉鎮不同環境。其次，對選定的社區進行初步的深度訪談，瞭解居民的活動習慣及對健康政策的認知。接著，利用 IPAQ-Taiwan 版本進行問卷調查，收集數據，包括居民的身體活動水平、社會支持及環境因素等。然後，根據數據結果進行社區的身體活動干預，譬如設置步道、舉辦運動活動，並對政策的實施效果進行長期跟蹤。此方法的關鍵在於將質性研究與量性評估相結合，不僅提供了有效的數據支持，還能針對社區的具體需求進行策略調整。此外，我們將利用數據可視化工具，提升政策相關者的理解與參與感，最終促進更有效的身體活動政策的制定。

實驗計畫

本研究的實驗計畫將分為幾個主要步驟：第一步，選擇台灣多個不同特徵的社區作為樣本，目標是涵蓋不同年齡、性別及社經背景的居民。第二步，進行內部焦點小組討論，以獲取居民對當前身體活動政策的看法及建議。第三步，分發 IPAQ-Taiwan 問卷，收集至少 500 名居民的數據，並運用統計分析方法進行回歸分析，評估各種因素對身體活動的影響。第四步，根據問卷結果制定針對社區的身體活動促進計畫，並實施為期六個月的干預。每月都將收集參與者的活動數據，最終進行效果評估，包括自我報告的活動水平、身體健康指標及居民的生活滿意度等。成功的指標包括居民身體活動水平的顯著提高、健康指標的改善及政策知識的增進，這將體現我們方法的有效性。

可信新穎度 — 最接近的真實論文

- Development and Validation of the Papua New Guinea Physical Activity Questionnaire (53.4% · airtiti.AL, 2024) — 區隔明確: 本研究專注於評估社區層面的身體活動政策有效性，並結合質性研究與量性評估，而該論文主要針對巴布亞新幾內亞的身體活動測量工具的開發，並未涉及政策評估或社區干預的實施。
- Development and Verification of Validity and Reliability of the International Physical Activity Questionnaire Taiwan Version (47.4% · airtiti.AL, 2004) — 區隔明確: 本研究使用 IPAQ-Taiwan 版本進行社區層面的身體活動評估，並結合社區干預措施，而該論文主要集中於 IPAQ 的發展與驗證，並未進行實際的社區評估或政策效果的分析。
- Self-affirmation enhances the intention to improve physical inactivity through health risk messages (46.9% · airtiti.AL, 2021) — 區隔明確: 本研究針對社區層面的身體活動政策進行系統性評估，並結合居民的社會支持及環境因素，而該論文則專注於自我肯定如何影響健康行為意圖，並未涉及社區政策或身體活動的實際評估。

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49. Reliability and validity of measures from the Behavioral Risk Factor Surveillance System
50. Physical activity and public health: A recommendation from the CDC and ACSM
51. Cognitive laboratory approach to designing questionnaires for surveys of the elderly

透過身體活動探討老年人在危機期間的心理韌性

後設資料

想法編號: seed-domain-3=>1=>2=>1

新穎度: 45.5%

最大相似度: 54.5% (近似於: “Factors associated with the recovery of activities of daily living after hospitalization for acute medical illness: a prospective cohort study”)

群集: 9

競賽得分: 3 勝

方法論契合度: 2/5

問題陳述

隨著全球疫情的爆發，老年人群體面臨著日益嚴重的心理健康挑戰，尤其是在封鎖和社交距離措施的影響下，許多老年人的身體活動量急劇下降。根據以往研究，身體活動被證明有助於提高心理健康，但在疫情這樣的危機期間，老年人如何保持身體活動並促進心理韌性仍是一個顯著問題。目前的文獻大多集中於疫情對心理健康的直接影響，而對身體活動的探討相對較少。因此，本研究旨在深入分析老年人身體活動的變化如何影響其心理健康的韌性，特別是在危機情形下的作用，這對於未來的健康政策和介入措施具有重要意義。

現有方法

目前的研究方法主要依賴於橫斷式研究，通過問卷調查和自報的身體活動量來評估心理健康狀況。這些方法的侷限性在於其只能提供時間點的數據，缺乏長期觀察的能力，無法捕捉到身體活動隨時間的變化及其對心理健康的持續影響。此外，許多研究忽略了社會支持和生活環境對身體活動與心理健康之間關係的調節作用，導致結論的片面性。

研究動機

由於現有的方法無法充分考慮到長期的身體活動變化和外部環境的影響，因此我們需要一種新的方法來追蹤這些變化及其與心理健康的關聯。這項研究的靈感來自於生態心理學，即環境與個體之間的交互作用。通過結合身體活動的客觀測量（例如使用穿戴設備）與心理健康的長期評估，我們希望能夠全面理解危機中老年人的心理韌性，進而設計出有效的介入方案。

提出方法

本研究將使用縱向設計，觀察參與者在 COVID-19 疫情期間的身體活動變化及其對心理健康的影響。具體方法如下：第一步，我們將招募一組社區中的老年人，使用可穿戴設備（如智慧手環）來實時監測其每日身體活動水平，這能提供更準確的數據，而不是單純依賴自報問卷。第二步，參與者將每月填寫心理健康評估問卷（例如 Geriatric Depression Scale、Anxiety Inventory），以評估其心理韌性指數。第三步，數據分析將採用重複測量方差分析（RM-ANOVA）來觀察身體活動與心理健康之間的長期關聯。另外，我們還將考慮社會支持和環境因素對該關聯性的調節作用，例如社區參與度和社交聯繫，這將使我們的模型更加全面。不僅要採集數據，還需進行質性訪談，以了解參與者的主觀體驗和挑戰。

實驗計畫

步驟一：招募老年參與者，選取年齡在 65 歲以上的社區成員，預計招募 150 名參與者。步驟二：發放智慧手環給參與者以收集他們的身體活動數據，並指導他們進行正確的佩戴和使用。步驟三：每月進行一次心理健康評估，包括 Geriatric Depression Scale 和 Anxiety Inventory 問卷。步驟四：設計一套社會支持調查問卷，分析參與者的社交聯繫和支援系統。步驟五：進行數據分析，使用重複測量方差分析來評估身體活動和心理健康之間的關聯性。成功的指標包括在研究期間內，參與者的身體活動水平有所上升且心理健康評估的分數有明顯改善，並顯示出社會支持的正面影響。

可信新穎度 — 最接近的真實論文

- Factors associated with the recovery of activities of daily living after hospitalization for acute medical illness: a prospective cohort study (54.5% · airtiti.AL, 2016) — 區隔明確: 該研究專注於急性醫療疾病住院後老年患者日常生活活動的恢復因素，而本提案則專注於疫情期間老年人的身體活動與心理健康韌性之間的關聯，這是一個不同的研究問題和方法。
- Deleterious effects of physical inactivity on the hippocampus: New insight into the increasing prevalence of stress-related depression (52.9% · airtiti.AL, 2015) — 區隔明確: 該研究探討的是身體不活動對海馬體的影響，並未直接關注老年人在疫情期間的身體活動與心理健康之間的關聯，而本提案則專注於老年人如何在危機期間通過身體活動來促進心理韌性。
- [The physical activity and life healing in psychiatric patients: taijiguan as an example]. (51.4% · airtiti.PubMed, 2014) — 區隔明確: 該研究主要針對慢性精神病患者的身體活動對健康的影響，而本提案專注於老年人群體在疫情期間的身體活動變化及其對心理健康的影響，這是一個不同的研究焦點。

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探討台灣老年人的健康信念與身體活動參與：混合方法研究

後設資料

想法編號: seed-theory-5=>2=>2=>3

新穎度: 40.6%

最大相似度: 59.4% (近似於: “[Comparison of Stages of Physical Activity Among Nurses, Physicians, and Allied Healthcare Professionals].”)

群集: 1

競賽得分: 2 勝

方法論契合度: 3/5

問題陳述

隨著台灣社會進入高齡化社會，老年人的健康問題日益受到關注。運動不足已被證明是導致多種健康問題的主要危險因素，然而，許多老年人卻因健康信念的影響而不積極參與身體活動。這些信念可能包括對於身體活動的誤解、對健康的恐懼、或是缺乏相關知識等。現有的研究多數集中於身體活動的量化測量，而對於老年人如何看待這些活動及其參與動機的探討卻相對較少。因此，有必要通過定量與定性研究的結合，深入挖掘影響老年人身體活動參與的內在因素，從而為未來的健康干預設計提供信息。

現有方法

目前的研究方法主要集中於量化評估身體活動的程度，比如使用國際身體活動量表等工具來衡量參與者的運動頻率和時間。然而，這些方法往往無法深入了解老年人對體育活動的內在信念和動機。雖然已有一些質性研究探討健康信念，但其樣本範圍和方法的多樣性仍有待提高，無法提供全面的了解。因此，現有的方法在探索老年人健康信念對活動參與影響方面顯得不足，特別是在文化特定的背景下，如台灣。

研究動機

現有方法的不足主要體現在其無法綜合量化數據和質性洞察來形成全面的理解。我們的研究靈感來自於人類行為科學的理論，尤其是健康信念模型。這些理論強調了信念、知識和社會因素如何影響個人的行為選擇。通過混合方法研究，我們可以更全面地探討影響老年人身體活動參與的動機及障礙，提供更具針對性的干預措施。這種方法的創新在於它不僅僅依賴於數據，而是通過深入的人類理解來創建更有效的健康行為促進策略。

提出方法

我們的研究將採用混合方法，結合質性和量化的數據收集技術。首先，我們將進行定性訪談，招募 50 位年齡在 65 歲以上的老年人，通過半結構化的訪談收集他們對身體活動的信念、感受和參與經歷。訪談將通過錄音進行，並使用主題分析法進行數據分析。其次，根據初步的質性研究結果，我們將設計一份調查問卷，量化收集 300 位老年人的健康信念、身體活動頻率、障礙與動機。問卷將包含例如健康信念量表、身體活動量表及社會支持評估問卷，以便更全面地捕捉參與者的健康行為模式。最終，通過將質性和量化數據結合，我們將能更好地理解老年人活動參與的內部和外部因素，並提出針對性的干預建議。

實驗計畫

本研究將進行為期六個月的實證研究。首先，我們將從三個不同的地區（城市、鄉村及部落地區）招募參與者，以確保樣本的多樣性。定性階段將包括至少 50 位參與者的訪談，預計每次訪談長度約為 60 分鐘，由經驗豐富的研究人員進行。質性數據將使用主題分析方法進行整理，主要集中在信念、動機和障礙等方面。隨後，我們將設計問卷並進行預試，預計收集 300 位老年人的填寫數據。調查數據將採用 SPSS 進行描述性統計和多元回歸分析，評估健康信念對身體活動參與度的影響。成功的標準將在於量化數據與質性研究結果的相互印證，以支持我們的研究假設。

可信新穎度 — 最接近的真實論文

- [Comparison of Stages of Physical Activity Among Nurses, Physicians, and Allied Healthcare Professionals]. (59.4% · airtiti.PubMed, 2016) — 區隔明確：本研究聚焦於台灣老年人的健康信念與身體活動參與，採用混合方法進行質性和量化的數據收集，而該論文主要比較不同醫療專業人員的身體活動階段，並未深入探討健康信念對身體活動的影響。
- Self-affirmation enhances the intention to improve physical inactivity through health risk messages (57.2% · airtiti.AL, 2021) — 區隔明確：本研究專注於台灣老年人的健康信念與身體活動參與，通過混合方法收集質性和量化數據，而該論文則探討自我肯定如何影響健康行為的意圖，並未針對老年人的健康信念進行深入分析。
- [Physical Activity and Quality of Life in Patients With Type II Diabetes Mellitus]. (56.8% · airtiti.PubMed, 2019) — 區隔明確：本研究針對台灣老年人的健康信念與身體活動參與進行探索，使用混合方法收集數據，而該論文則專注於 2 型糖尿病患者的身體活動狀況與生活品質，並未探討健康信念的影響。

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42. Reliability and validity of measures from the Behavioral Risk Factor Surveillance System
43. Physical activity and public health: A recommendation from the CDC and ACSM
44. Cognitive laboratory approach to designing questionnaires for surveys of the elderly

評估脆弱群體在身體活動水平中的決定因素：以 IPAQ 為基礎的調查

後設資料

想法編號: seed-theory-3=>1=>3=>2

新穎度: 39.3%

最大相似度: 60.7% (近似於: “Development and Validation of the Papua New Guinea Physical Activity Questionnaire”)

群集: 3

競賽得分: 2 勝

方法論契合度: 5/5

問題陳述

目前，身體活動不足被認為是影響健康的重要因素，特別是在兒童和青少年中。有哮喘的兒童在身體活動的參與上常面臨挑戰，這不僅影響他們的身體健康，也可能影響心理健康。女性青少年尤其易受到社會文化因素的影響，可能導致她們的身體活動水平低下。然而，現有的研究對於這些脆弱群體身體活動的決定因素尚未有深入的探討。此外，自我報告的數據可能存在偏差，無法真實反映其身體活動的狀況，因此亟需一個針對這些特定群體的系統性研究來填補這一空白。

現有方法

現有的研究主要採用自我報告問卷進行身體活動的評估，如 IPAQ 的長版及短版，但這些方法經常受到社會期望偏差及記憶限制的影響。此外，對於有特殊健康狀況如哮喘的兒童或女性青少年，研究缺乏針對性，未考慮其獨特的心理和環境因素，因此無法準確描繪出影響其身體活動的全貌。這些方法在客觀測量方面的不足，使得結果的可用性和有效性受到質疑。

研究動機

由於現有方法的限制，特別是在針對脆弱群體的個人及環境因素的理解不足，本研究提案將採用結合自我報告與環境評估的混合方法進行深入分析。透過與哮喘專家及心理學家合作，將個人心理因素納入考量，並利用地理信息系統 (GIS) 來評估環境因素，以期更全面地理解影響身體活動的決定因素。這將有助於設計更具針對性的介入措施，以促進這些群體的身體健康。

提出方法

本研究將採取混合方法，即將自我報告的 IPAQ 與客觀測量工具相結合。計畫的初步步驟包括：1. 選擇樣本：招募合適的參與者，包括有哮喘的兒童與女性青少年，預計參與者數量為 500 人。2. 問卷調查：發放經翻譯和驗證過的 IPAQ 問卷，以了解他們的身體活動水平，同時收集相關的個人特徵。3. 心理因素評估：使用標準化的心理量表，評估參與者的自我效能、焦慮和抑鬱水平。4. 環境因素評估：運用 GIS 技術，分析參與者生活周邊的環境特徵，包括綠地分佈、運動設施的可達性等。5. 數據整合與分析：將收集的數據進行統計分析，使用多元回歸模型評估各因素對身體活動的影響程度，並據此制定針對性的介入措施。通過這些步驟，我們將得出對於身體活動水平的綜理解，並力求探索出適合不同脆弱群體的健康促進策略。

實驗計畫

實驗計劃將分為以下幾個步驟：1. 樣本選擇：針對有哮喘的兒童和女性青少年，從醫療機構和學校中招募 500 名參與者。2. 問卷調查：發放 IPAQ 問卷及心理量表，收集自我報告的身體活動數據與心理狀態。3. 環境數據收集：使用 GIS 技術，收集參與者居住區域的環境資料。4. 數據分析：利用 SPSS 或 R 進行數據分析，評估不同因素對身體活動的影響，使用如多元回歸分析等統計方法。5. 成功指標：若參與者的身體活動水平有顯著提升且心理健康狀況改善，則本研究將被視為成功，並可為未來的健康介入提供實證支持。

可信新穎度 — 最接近的真實論文

- Development and Validation of the Papua New Guinea Physical Activity Questionnaire (60.7% · airiti.AL, 2024) — 區隔明確：本研究專注於台灣的脆弱群體（如有哮喘的兒童和女性青少年）及其身體活動的決定因素，並結合客觀測量工具與心理和環境因素的評估，而該研究則針對巴布亞新幾內亞的整體人群進行身體活動工具的開發與驗證，缺乏對特定脆弱群體的深入分析。
- Reliability and Validity of a Questionnaire for Assessment of Energy Expenditure and Physical Activity in Epidemiological Studies (57.4% · airiti.AL, 1998) — 增量：本研究將使用混合方法結合自我報告的 IPAQ 和客觀測量工具，並評估心理和環境因素對身體活動的影響，而該研究僅開發了一種自我報告的問卷來評估能量消耗和身體活動，缺乏對心理和環境因素的考量。
- Self-affirmation enhances the intention to improve physical inactivity through health risk messages (56.3% · airiti.AL, 2021) — 區隔明確：本研究針對特定脆弱群體的身體活動決定因素進行系統性研究，並使用 GIS 技術分析環境因素，而該研究則專注於自我肯定如何影響健康行為意圖，並未針對身體活動的具體決定因素進行探討。

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數位自我效能介入提升後心肌梗塞患者身體活動遵從性之研究提案

後設資料

想法編號: seed-domain-5=>1=>1=>2

新穎度: 36.4%

最大相似度: 63.6% (近似於: “Self-affirmation enhances the intention to improve physical inactivity through health risk messages”)

群集: 0

競賽得分: 1 勝

方法論契合度: 4/5

問題陳述

後心肌梗塞患者在康復過程中，進行規律的身體活動對其健康復甦極為重要。然而，許多患者難以遵從醫療建議，導致身體活動的參與程度不足，進而影響康復效果及生活品質。研究顯示，這部分患者的自我效能普遍較低，對於運動的信心不足，無法有效激勵自身在恢復期間提高體力活動水平。此外，現有的干預措施多為傳統面對面的諮詢或健身計畫，缺乏針對患者個別需求的定制化以及數位化支持。因此，開發一項結合數位技術與自我效能策略的干預計畫，將為後心肌梗塞患者創造更好的運動遵從性、改善健康結果，這是一項具有重大的臨床意義的挑戰。

現有方法

目前對於後心肌梗塞患者的運動介入主要依賴傳統方法，例如醫護人員的面對面指導和健身房的集體課程，這些方法的效果有限。傳統的運動計畫通常忽略了患者的個別需求與心理狀態，並未考慮到患者在自我效能方面的差異。根據過往研究，這些傳統方法無法有效提升患者的自信心和運動動機，進而導致長期遵從性不佳。此外，現行的數位應用程序大多數僅提供運動建議，缺少針對自我效能的策略。因此，現在的介入方法未能充分利用數位科技的潛力來改善患者的運動遵從性。

研究動機

現有的運動介入方法未能有效提高後心肌梗塞患者的身體活動遵從性，主要是因為它們缺乏個性化和對自我效能的重視。我們的提案將融合數位工具和自我效能策略，這樣不僅能夠提供個性化的運動計畫，還能利用數位技術提供即時反饋和鼓勵。我們相信，通過幫助患者建立運動自信和習慣，並隨時跟踪進展，將能夠顯著改善他們的身體活動水平。這種方法提供了一種全新的思路，能夠有效應對傳統方法的局限性，並正向促進患者的身心健康。

提出方法

本提案的核心是開發一個數位自我效能介入平台，專為後心肌梗塞患者設計。整個方法將分為以下幾個步驟：首先，對參與者進行初步評估，利用國際身體活動量表（IPAQ）來測量其目前的身體活動狀況和自我效能水平。接著，系統將根據個別的健康狀況和需求，提供個性化的身體活動計畫。這些計畫將以遊戲化的方式呈現，增加參與者的興趣和參與度。此外，系統會整合行為心理學的策略，如設定小目標、實時反饋和社區支持，以增強參與者的自信心和動力。介入過程中，系統將定期發送鼓勵性消息和進度報告，幫助患者維持運動習慣。最後，我們將使用持續的評估工具來測量介入效果，包括運動遵從率、心血管健康指標及參與者的日常生活品質。

實驗計畫

本研究將招募 60 名後心肌梗塞患者，進行為期 12 周的實驗。參與者將隨機分為介入組和對照組。介入組將使用數位平台，而對照組則將接受傳統的運動指導。在實驗開始前，所有參加者將完成 IPAQ 評估，並列出個人的運動目標。隨後，介入組每日使用數位平台，進行個性化的運動計畫，並每周接受一次進度檢查。對照組則根據傳統運動計畫進行，並經常性接受醫護人員的面對面指導。實驗結束後，我們將再次使用 IPAQ 評估參加者的身體活動遵從性，並通過心血管健康檢查來評估實驗效果。評估指標包括運動遵從率、心血管健康指標變化及生活品質評價。成功的標誌將是在介入組中觀察到顯著的身體活動水平提升及心血管健康的改善。

可信新穎度 — 最接近的真實論文

- Self-affirmation enhances the intention to improve physical inactivity through health risk messages (63.6% · airtiti.AL, 2021) — 區隔明確: 本提案專注於後心肌梗塞患者的數位自我效能介入，旨在提升他們的身體活動遵從性，而該研究則探討自我肯定對於改善健康行為意圖的影響，並未針對特定患者群體或使用數位平台進行干預。
- Exercise Participation, Self-esteem and Body Image among College Students (51.4% · airtiti.AL, 2010) — 區隔明確: 本提案針對後心肌梗塞患者的身體活動遵從性進行數位化介入，而該研究則主要關注大學生的自尊和身體形象，並未涉及心血管疾病患者或數位介入的使用。
- The effect of intervention behavior change strategies for promote physical activity volume in sedentary peoples (50.9% · airtiti.AL, 2021) — 區隔明確: 本提案專注於後心肌梗塞患者的個性化數位介入，並使用遊戲化和行為心理學策略來提升運動遵從性，而該研究則針對坐式生活者的行為改變策略進行介入，且樣本量和研究設計不同。

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26. Assessment of physical activity in epidemiologic research: Problems and prospects
27. Construct validity in physical activity research
28. Moderate leisure-time physical activity: who is meeting the public health recommendations?
29. Use, misuse and abuse of questionnaires on quality of life
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33. A validation of two motion sensors in the prediction of child and adult physical activity levels

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36. Physical activity and public health: A recommendation from the CDC and ACSM
37. Cognitive laboratory approach to designing questionnaires for surveys of the elderly

早期教育環境對學齡前兒童身體活動與發展的影響研究

後設資料

想法編號: seed-theory-3=>2=>2=>1

新穎度: 38.9%

最大相似度: 61.1% (近似於: “幼兒身體活動的時間運用和發展狀況”)

群集: 4

競賽得分: 1 勝

方法論契合度: 2/5

問題陳述

隨著科技的進步和城市化的發展，學齡前兒童的身體活動水平正在逐漸下降，這導致了健康危機的出現，包括肥胖率的上升和生活方式疾病的增多。幼兒園是幼兒日常生活的重要場域，教師的體育活動態度和實施的體能活動計劃對於促進兒童身體活動至關重要。然而，目前對於學齡前教師如何影響兒童身體活動的研究仍然不足，且缺乏針對結構化體能活動計劃的實證研究。此外，雖然已有一些研究探討了身體活動對健康的影響，但缺乏專門針對台灣學齡前兒童及其教育環境的系統分析。因此，本研究將針對上述問題進行深入探討，以期改善學齡前兒童的健康和發展。

現有方法

目前的研究方法主要集中在觀察和問卷調查教師對於體能活動的態度，然而這些方法往往無法提供客觀的體能活動數據，且缺乏對教師行為實施的系統評估。例如，雖然國際身體活動量表為測量身體活動提供了一些工具，但這些工具普遍未能充分考慮文化和社會背景的差異，可能導致結果的局限性。此外，許多研究未能探索教師的態度如何實際轉化為有效的教學實踐。因此，現有方法在理解教師對兒童身體活動影響方面的能力仍顯不足。

研究動機

由於現有方法過於依賴自我報告和觀察性數據，難以全面了解教師的實際行為以及這些行為對兒童的具體影響，本研究計劃採用混合方法，結合教師調查、兒童實際身體活動的客觀測量和結構化的體能活動計劃。這種創新的方法將能更有效地揭示教師態度與兒童健康結果之間的實際關係，並提供實證支持，以制定更加有效的體能活動干預措施。我們相信，通過這種綜合性的方法，可以更全面地捕捉到學齡前教育環境對兒童發展的影響。

提出方法

本研究將採用混合方法進行研究，首先設計一份針對幼兒園教師的問卷，內容涵蓋對體育活動的態度、認知及其在教學中的具體實踐。此外，將引入動態監測技術，以客觀測量參與研究的學齡前兒童的實際身體活動水平，這將透過佩戴加速器來實現。同時，設計為期六週的結構性體能活動計劃，計劃包括多種活動類型（如體操、舞蹈和運動遊戲），目的是在日常課程中增強兒童的運動技能。數據收集階段將包括問卷調查的定量數據與兒童活動數據的質量分析。接著，將透過焦點小組討論來獲取教師對這些活動實施的看法與建議，進一步驗證和完善量化數據的解釋。最終通過數據分析，將形成一個基於實證的模型，展示教師的體育態度如何影響兒童的身體活動和健康結果。

實驗計畫

實驗計畫將分為幾個步驟：首先，選取三所台灣不同地區的幼兒園作為研究對象，並隨機選取 60 名教師進行問卷調查。問卷涵蓋教師對體育活動的態度及其具體教學實踐。其次，招募 100 名學齡前兒童，要求其佩戴加速器以客觀評估他們的身體活動水平。然後，實施為期六週的體能活動計畫，參與的教師將在教學中融入這些活動，並收集兒童的運動技能表現數據。研究將使用 SPSS 進行數據分析，包括描述性統計、相關性分析和多元回歸，評估教師態度、兒童活動量與身體發展之間的關係。評估成功的標準包括教師的積極態度預測兒童的活動水平提升，以及在結構化活動後兒童運動技能的顯著改善。

可信新穎度 — 最接近的真實論文

- 幼兒身體活動的時間運用和發展狀況 (61.1% · airtiti.AL, 2020) — 區隔明確: 本研究專注於學齡前兒童的身體活動與教師的體育活動態度之間的關係，並且採用動態監測技術來客觀測量兒童的身體活動水平，而該文獻主要探討的是幼兒身體活動的時間運用與發展狀況，並未涉及教師的影響或結構化體能活動計畫的實證研究。
- COMPARISON OF LEVELS OF DAILY PHYSICAL ACTIVITY BETWEEN JAPANESE PRESCHOOL CHILDREN ENROLLED IN KINDERGARTENS AND NURSERY SCHOOLS (59.0% · airtiti.AL, 2009) — 區隔明確: 本研究針對台灣學齡前兒童的身體活動進行系統分析，並設計了具體的結構性體能活動計畫，而該研究則比較了日本幼兒在不同教育機構中的日常身體活動水平，缺乏針對教師影響的深入探討。
- 【論文摘要】Investigation Of The Different Factors Linked To The Sedentary And Physical Inactivity Behaviours Of Children And Adolescents In France (56.2% · airtiti.AL, 2023) — 區隔明確: 本研究專注於台灣學齡前兒童的身體活動及其教育環境的影響，並使用混合方法進行深入分析，而該研究則探討了法國兒童和青少年久坐及身體不活動行為的不同因素，缺乏針對特定年齡群體和教育環境的系統性研究。

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43. Cognitive laboratory approach to designing questionnaires for surveys of the elderly

跨文化與年齡適應的身體活動評估工具的發展與驗證

後設資料

想法編號: seed-domain-4=>3=>2=>3

新穎度: 46.2%

最大相似度: 53.8% (近似於: “Development and Validation of the Papua New Guinea Physical Activity Questionnaire”)

群集: 6

競賽得分: 1 勝

方法論契合度: 2/5

問題陳述

身體活動不足已被廣泛認識為全球健康的重要威脅，特別是在老年人和兒童之中。雖然目前已有多種身體活動評估工具，但這些工具通常針對特定年齡群體或文化背景，缺乏跨文化的適應性和各年齡層的靈活性。因此，無法有效比較不同文化或年齡群體的身體活動水平，這限制了公共健康政策的制定和實施。同時，傳統評估工具未能充分考慮到語言和文化的差異，導致評估結果的可靠性和有效性受到影響。因此，開發一個能夠針對不同年齡群體並且具備文化適應性的身體活動評估工具是解決這一問題的必要舉措。

現有方法

目前存在的身體活動評估方法包括國際身體活動問卷 (IPAQ)、健康活動問卷 (HAQ) 和其他針對特定年齡群體的工具。然而，這些工具往往存在以下幾個局限性：首先，許多工具僅針對一個年齡段，例如專為老年人或兒童設計，缺乏針對跨年齡段的通用性；其次，這些工具可能未能充分考慮到文化差異，導致在不同文化背景下的效度不足；最後，部分工具的自我報告依賴程度較高，可能引發回憶偏差，因此實際活動水平的評估可能不夠準確。

研究動機

現有的身體活動評估工具未能滿足跨文化及不同年齡群體的需求，這不僅限制了對體能活動的準確評估，還對公共健康政策的制定造成困難。根據之前的研究，文化和年齡會影響身體活動的方式與頻率，因此需要一個綜合考量這些因素的工具。我們的靈感來自於將老年人和兒童的活動特徵融合進一個統一的評估工具，這樣的工具能夠提供更全面的信息，有助於促進各年齡層的健康行為。

提出方法

本研究旨在開發並驗證一個兼具跨文化及年齡適應性的身體活動評估工具，具體步驟如下：首先，將針對台灣文化進行文獻回顧，了解當地的身體活動習慣及用語，並設計多個評估項目，以符合不同年齡段的需求。其次，運用專家審查、焦點小組及初步田野研究收集意見，進行工具的初步設計。然後，通過翻譯與回翻的方法，確保語言的準確性與適用性。接下來，將進行信度與效度的檢驗，選取多元文化及年齡層的受試者進行測試。最後，利用心理測量學方法評估工具的可靠性，並進行儀器的調整和優化，以確保最佳的使用效果。

實驗計畫

實驗計畫將分為幾個步驟：首先，招募來自各個年齡群體（兒童及老年人）的參與者，並確保具有文化多樣性。選擇的樣本將涵蓋不同社會經濟背景。其次，進行初步的工具測試，並收集數據以評估工具的有效性和可靠性，這包括使用問卷調查和可穿戴設備的數據。第三，進行數據分析，利用統計學方法評估工具的信效度，包括 Cronbach's α 測試及結構方程模型。評價指標包括參與者的身體活動水平、滿意度及使用經驗。理想的結果是該工具能夠在不同年齡段及文化中顯示出良好的適用性及準確性，並能有效反映參與者的身體活動狀況，從而為公共健康政策提供依據。

可信新穎度 — 最接近的真實論文

- Development and Validation of the Papua New Guinea Physical Activity Questionnaire (53.8% · airtiti.AL, 2024) — 區隔明確：該研究專注於巴布亞新幾內亞的身體活動測量工具，並未考慮跨文化和年齡適應性，而本提案旨在開發一個能夠適應不同文化和年齡群體的身體活動評估工具，這是其根本的不同之處。
- Reliability and Validity of a Questionnaire for Assessment of Energy Expenditure and Physical Activity in Epidemiological Studies (53.4% · airtiti.AL, 1998) — 區隔明確：該研究的問卷主要針對 18 至 64 歲的成年人進行能量消耗和身體活動的評估，而本提案則針對不同年齡層（包括兒童和老年人）及跨文化背景進行設計，這使得兩者的研究範圍和目標人群存在顯著差異。
- Management of falls in community-dwelling older adults: clinical guidance statement from the Academy of Geriatric Physical Therapy of the American Physical Therapy Association. (51.1% · airtiti.PubMed, 2015) — 區隔明確：該研究主要集中於老年人跌倒的管理，並未涉及身體活動評估工具的開發，而本提案的重點是創建一個針對不同年齡和文化的身體活動評估工具，這使得研究問題和方法上存在根本的不同。

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結合社交參與與身體活動評估的高齡照護雙重評估框架

後設資料

想法編號: seed-domain-1=>1=>2=>3

新穎度: 46.9%

最大相似度: 53.1% (近似於: “Comparison of Nutritional Habits, Physical Activity Levels and Quality of Life among Normal Cognition Elderly Individuals Living in Nursing Homes or at Their Residence”)

群集: 2

競賽得分: 0 勝

方法論契合度: 4/5

問題陳述

在台灣，隨著老年人口的快速增長，老年人的健康照護已成為公共衛生的重大挑戰。研究顯示，身體活動量不足與社交孤立直接影響老年人的功能健康狀況。然而，現有的健康評估多數專注於單一方面，如身體活動或社交互動，缺乏多維度的評估視角。這樣的模型未能全面捕捉老年人的健康狀態，且難以為個性化治療計劃提供必要的資訊。實際上，不同社交參與的程度可能會影響身體活動的持續性，因此，我們需要一種既能量化身體活動，又能評估社交參與的評估工具，以促進老年人的自主性與生活品質，這便是我們提出此研究的背景與重要性所在。

現有方法

目前在台灣的身體活動評估大多依賴於國際身體活動量表 (IPAQ)，其有效性已得到一定的認可。然而，這些方法僅針對身體活動進行測量，忽視了社交互動對老年人健康的影響。另外，許多研究已經證實了社交參與與健康之間的正相關，但目前缺乏有效集成兩者的評估工具。因此，現有的評估方法無法充分捕捉老年人的整體健康狀態，這造成在制定針對性的健康促進干預時的困難。

研究動機

隨著老年人健康問題的複雜化，傳統的單一維度評估方法顯然不足以解決當前面臨的挑戰。我們的靈感源於社會心理學中社交參與對健康影響的研究，且已知身體活動不僅能改善生理健康，亦能促進社會交往和心理健康。整合這兩種評估工具，能夠更全面地了解老年人的健康狀況，並為設計個性化的健康促進計劃提供依據。因此，我們有理由相信新提出的雙重評估框架將有效超越現有方法，為老年人提供更好的健康照護與干預策略。

提出方法

本研究提出的雙重評估框架旨在融合 IPAQ 身體活動量表和社交參與及自我效能工具，並通過以下步驟實施：首先，對台灣老年人口進行背景調查，包括社會經濟狀況、生活方式、健康狀態等。接下來，將採用 IPAQ 評估三種身體活動模式的參與程度。其次，使用自我效能量表來評估老年人的社交互動能力和參與程度。本研究將設計一個數據收集平台，結合線上問卷和社交媒體互動，進一步提高參與度。分析將運用結構方程模型來探討身體活動和社交參與對老年人功能健康的影響。最終，該框架旨在為臨床工作者提供一個多元評估工具，進而制定針對性的健康促進計劃，以提升老年人的生活質量與健康狀態。

實驗計畫

本研究將進行系列實驗步驟，以檢驗我們的雙重評估框架的有效性。首先，招募 300 名 65 歲以上的老年參與者，並收集他們的基本資料，如性別、年齡、健康狀況等。參與者將填寫 IPAQ 評估問卷以及社交參與和自我效能量表。為確保數據的可靠性，我們將進行預測試，並基於這些數據來進行結構方程模型的構建。隨後，將根據參與者的身體活動和社交參與得分，分組並進行長期的身體活動干預（例如，社交健身小組活動）。最後，通過與基線測量進行比較，使用如身體健康指數、生活滿意度調查等指標進行效果評估。成功的指標包括身體活動增加、社交參與度提升及生活質量的改善，這些將在最終的結果分析中進行報告。

可信新穎度 — 最接近的真實論文

- Comparison of Nutritional Habits, Physical Activity Levels and Quality of Life among Normal Cognition Elderly Individuals Living in Nursing Homes or at Their Residence (53.1% · airtiti.AL, 2019) — 區隔明確: 本研究專注於結合社交參與與身體活動的雙重評估框架，而該論文僅比較了居住在護理機構與居家老年人的營養習慣、身體活動水平和生活質量，缺乏對社交參與的評估。
- Reliability and Validity of a Questionnaire for Assessment of Energy Expenditure and Physical Activity in Epidemiological Studies (52.0% · airtiti.AL, 1998) — 區隔明確: 本研究的雙重評估框架不僅評估身體活動，還包括社交參與和自我效能的測量，而該論文僅開發了一個針對能量消耗和身體活動的問卷，未考慮社交因素。
- Development and Validation of the Papua New Guinea Physical Activity Questionnaire (51.9% · airtiti.AL, 2024) — 區隔明確: 本研究提出的框架將身體活動和社交參與結合進行綜合評估，而該論文專注於巴布亞新幾內亞的身體活動測量工具的開發，並未探討社交參與對老年人健康的影響。

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